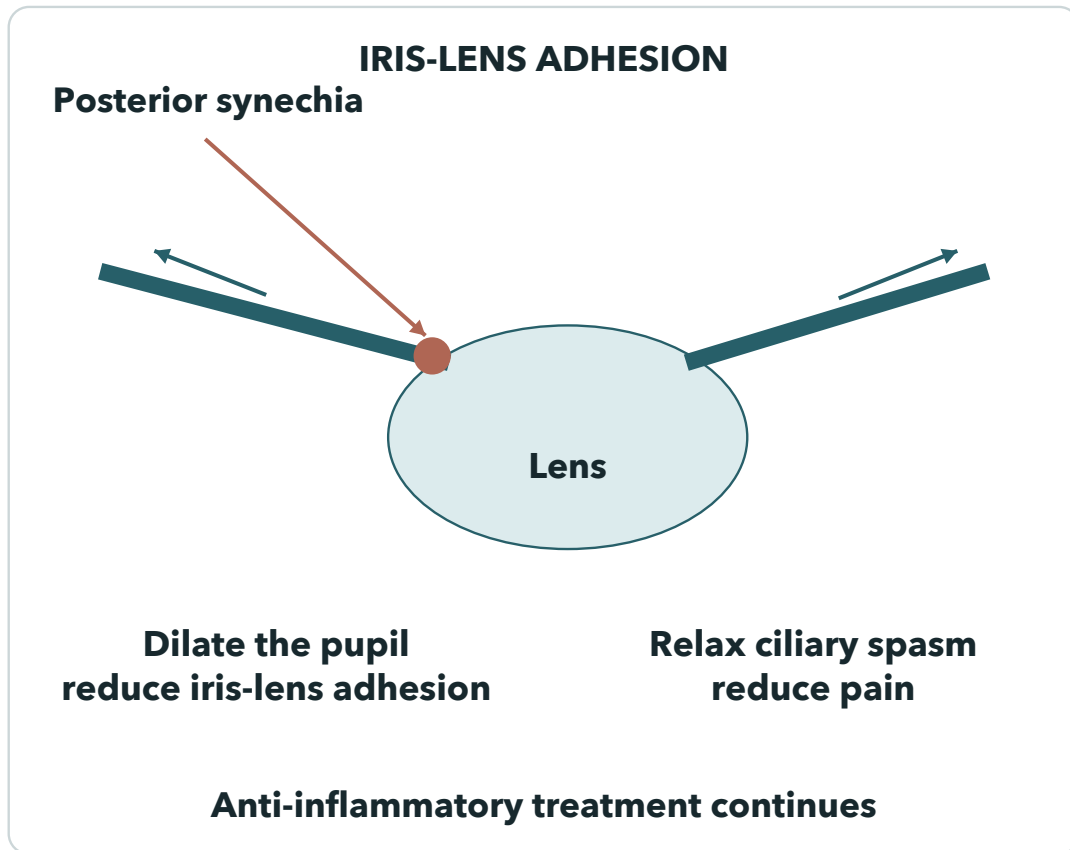


Anterior uveitis: phenotype before panels

Use the pupil, pressure and systemic history to guide the next investigation.

Cycloplegia has two useful effects

A | Pain and iris-lens adhesion



Original anterior-segment section. Mydriasis helps reduce adhesion; cycloplegia relaxes painful ciliary spasm.

Examination anchors

Finding	Why it matters
Cells and flare	Slit-lamp evidence of chamber inflammation; document response over time.
Posterior synechiae	Iris adheres to lens; recent adhesions may release with dilation, while established ones may persist.
Clear epithelium	Does not by itself exclude infectious uveitis.
Pressure spike and iris atrophy	Recurrent unilateral disease with these signs suggests a viral aetiology.

References AAO EyeWiki | Acute anterior uveitis | AAO EyeWiki | Herpes simplex uveitis

Choose tests that can change care

B | Three directed routes

RECURRENT ACUTE + BACK / ENTHESIS SYMPTOMS
HLA-B27 and spondyloarthritis assessment

UNILATERAL + HIGH IOP + IRIS ATROPHY
Consider viral aetiology and aqueous PCR

GRANULOMATOUS / BILATERAL / SYSTEMIC CUES
Directed sarcoid, TB and other assessment

TREATABLE MIMICS MATTER
Syphilis screening; avoid undirected panels

Original phenotype-guided investigation map. Testing accompanies treatment and is tailored to the patient and local epidemiology.

Targeted investigation

Recurrent acute episodes with inflammatory back pain or enthesitis symptoms warrant HLA-B27 and spondyloarthritis assessment. Suspected hypertensive viral uveitis may need aqueous PCR for HSV, VZV or CMV through the specialist service. Syphilis is an important treatable mimic; tuberculosis, sarcoidosis and other testing depend on risk and phenotype. Avoid assigning causation from an isolated low-positive broad-panel result. Cycloplegia complements anti-inflammatory therapy, and infectious disease may require cause-directed treatment as well. When a patient reports morning back stiffness that improves with activity, ask about enthesitis, skin and bowel symptoms. When the clues instead are pressure spikes and sectoral iris atrophy in one eye, viral investigation becomes more relevant than assuming a generic autoimmune label.